

Policy on Review of Medical Records

«Hospital Name»

Document control

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Date created	_____	Date of implementation	_____
Prepared by	Medical Records In-Charge Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

Statement of intent

«Hospital Name» carries out a periodic review of medical records, so gaps are found, documented and corrected before they compromise patient care.

1. Purpose

This policy sets out how «Hospital Name» reviews medical records periodically, using a representative sample, conducted by identified individuals, based on identified parameters, covering both active and discharged patients, documenting deficiencies found, and acting on them.

This policy does not cover medical record content, document control, or confidentiality and security of records — those are covered in other hospital policies.

2. Scope

This policy applies to the periodic review of physical and/or electronic medical records at «Hospital Name», covering both active and discharged patients.

3. Policy standards

The medical records at «Hospital Name» are reviewed periodically.

The review uses a representative sample based on statistical principles.

The review is conducted by identified individuals.

The review of records is based on identified parameters.

The review process includes records of both active and discharged patients.

The review points out and documents any deficiencies in records.

Appropriate corrective and preventive measures are undertaken.

4. Non-negotiable rules

1. Do not leave medical records unreviewed — the organisation must define and follow a periodicity for review.
2. Do not draw a review sample outside a defined statistical sampling principle — for example simple random or systematic random sampling — based on total discharges (including deaths) and total in-patients.

3. Do not let anyone conduct a medical record review without being identified and authorised for it by the organisation.
4. Do not review records without identified parameters — at a minimum, timeliness, legibility and completeness must be covered.
5. Do not limit the review to only active or only discharged patients — an adequate mix of both must be used.
6. Do not leave a deficiency found during review undocumented.
7. Do not leave a documented deficiency without a corrective action carried out in a defined time, or without disseminating the resulting preventive action to relevant staff.
8. Staff who see a rule under this policy broken report it the same shift to the Medical Records In-Charge or the Medical Superintendent.

5. What we do

5.1 Periodic review

«Hospital Name» defines the periodicity of medical record review. A checklist may be used for this purpose. The review is carried out for physical and/or electronic medical records.

5.2 Representative sampling

«Hospital Name» defines the principles on which its review sample is based — for example simple random or systematic random sampling. The review is based on total discharges, including deaths, and total in-patients.

5.3 Identified reviewers

«Hospital Name» identifies and authorises the individuals who conduct the medical record review.

5.4 Review parameters

At a minimum, the review at «Hospital Name» covers timeliness, legibility and completeness of the medical records. Other parameters that may be included are the completeness of consent forms and the availability of operation or procedure notes. A checklist may be used for this purpose. The review points out and documents deficiencies — for example a final diagnosis not mentioned, incomplete operation theatre notes in an operated patient, incomplete consents, or a missing signature, name, date or time for an entry.

5.5 Active and discharged patients

The review process at «Hospital Name» includes an adequate mix of records of both active and discharged patients.

5.6 Documenting deficiencies

«Hospital Name» points out and documents any deficiencies found during the review — for example a missing final diagnosis or absence of operation theatre notes in an operated patient.

5.7 Corrective and preventive action

Based on documented deficiencies, «Hospital Name» carries out appropriate corrections within a defined time, and documents them. Preventive actions arising from the review are disseminated to the relevant staff.

6. Governance and responsibility

Role	Responsibility
Medical Records In-Charge	Owens this policy. Coordinates the review schedule, sampling, and CAPA tracking.
Identified Reviewers	Conduct the medical record review against defined parameters.
Medical Superintendent	Oversees implementation of this policy. Receives escalations under Section 4.

All Staff	Correct identified deficiencies in their entries. Report identified risks and rule breaches.
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7. Quality monitoring

The Medical Records In-Charge reviews the review programme itself — sample adequacy, reviewer authorisation, and CAPA closure — and reports findings to the Medical Superintendent. This policy is reviewed annually, or sooner if NABH standards change.

8. Training and staff acknowledgement

Identified reviewers and relevant staff are trained on the review parameters and the corrective/preventive action process described in Section 5, and sign the acknowledgement below.

Acknowledgement statement:

"I have read the Policy on Review of Medical Records of «Hospital Name». I will follow the processes described."

Signature table (one row per staff member):

Staff Name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

9. Distribution

This policy is distributed to the Medical Records department, all clinical department heads, and is made available to all staff through the hospital intranet and department policy folders.

10. Abbreviations

Abbreviation	Meaning
HCO	Hospital (Full Accreditation programme under NABH Hospitals 6th Edition)
NABH	National Accreditation Board for Hospitals and Healthcare Providers
IMS	Information Management System (NABH Hospitals 6th Edition chapter)
OE	Objective Element

11. Traceability to NABH HCO Full Accreditation 6th Edition — IMS.7

This table is an index. It is not how the process is organised. An asterisk in the Level column means documentation of the process is required.

OE	Level	Requirement	Where addressed	Responsible
IMS.7.a	CORE	Medical records reviewed periodically	Section 5.1	Medical Records In-Charge
IMS.7.b	Commitment	Review uses representative sample	Section 5.2	Medical Records In-Charge
IMS.7.c	Commitment	Review conducted by identified individuals	Section 5.3	Identified Reviewers

IMS.7.d	Commitment	Review based on identified parameters	Section 5.4	Identified Reviewers
IMS.7.e	Commitment	Review includes active and discharged patients	Section 5.5	Identified Reviewers
IMS.7.f	Commitment	Review documents deficiencies	Section 5.6	Identified Reviewers
IMS.7.g	Commitment	Corrective/preventive measures undertaken	Section 5.7	Medical Records In-Charge

12. Required Records / Evidence Checklist

IMS.7.a — Periodic review.

- Defined review-periodicity document and review checklist, covering physical and/or electronic records.

IMS.7.b — Representative sampling.

- Defined sampling-principle record and sample-size calculation based on total discharges/in-patients.

IMS.7.c — Identified reviewers.

- Authorisation record naming the individuals conducting the review.

IMS.7.d — Review parameters.

- Review-parameter checklist covering timeliness, legibility, completeness, and any additional parameters used.

IMS.7.e — Active/discharged mix.

- Sample-composition record confirming an adequate mix of active and discharged patient records.

IMS.7.f — Documented deficiencies.

- Deficiency log from each review cycle.

IMS.7.g — CAPA.

- Corrective-action record with defined completion time.
- Preventive-action dissemination record to relevant staff.

13. References

- NABH Guidebook for Accreditation of Hospitals, 6th Edition — Standard IMS.7, "The organisation carries out a review of medical records."

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked _____ must be completed before the document is issued.

The requirements in this document are accreditation requirements of the NABH Full Accreditation Standards for Hospitals, 6th Edition, not duties under a named Act of Parliament. Statutory duties that arise under other documents of «Hospital Name» remain those documents. «Hospital Name» is responsible for verifying any statutory duty that applies to it; this document does not constitute legal advice.

The clinical and technical content reflects recognised national and international guidance current at the date of preparation. «Hospital Name» remains responsible for verifying that it is current and consistent with the edition of the accreditation standard against which it is being assessed.

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